

Brad Pitt's Return to Drinking Is Not Unsafe, Experts Say, but It's Not for Everyone

His disclosure in an interview revived a key question about sobriety: whether someone who once drank dangerously can ever drink moderately.



By Mark Walker

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Brad Pitt went seven years without a drink. Then he started again — carefully, he said, and on his own terms.

He mentioned it almost in passing in an Esquire interview published this week: Mr. Pitt had gotten “back off the wagon” and now drinks “in a more restrained manner.” Never in large quantities, he insisted. He had gotten overconfident a couple of times and pulled back.

His comments stirred up one of the oldest and least settled arguments about sobriety: whether someone who once drank dangerously can ever drink safely again.

Mr. Pitt quit drinking in 2016, the year his marriage to the actress Angelina Jolie collapsed. After she filed for divorce, he spent a year and a half in Alcoholics Anonymous and has credited the program with getting him sober. “I had taken things as far as I could take it, so I removed my drinking privileges,” he told The New York Times in 2019.

Alcoholics Anonymous focuses on abstinence-based recovery from alcoholism and does not comment publicly about members. A representative from the A.A. General Service Office said only that “members of Alcoholics Anonymous have only one purpose: to stay sober ourselves and help other alcoholics to achieve sobriety.”

It was his history with A.A. that touched a nerve among people who have fought to stay sober, or have friends or relatives who have struggled with recovery. People who figuratively followed Mr. Pitt into a room of folding chairs learned of his exit from it.

Experts and researchers said the reactions split along lines that had less to do with Mr. Pitt than with how the people reacting had gotten sober themselves.

Mr. Pitt, during his interview, offered himself as no one's model for sobriety. He described where he was and claimed nothing about where he will be later.

Researchers said that account was less unusual than it sounds.

"People with an alcohol disorder show a variety of different paths through which they resolve that alcohol use disorder," said Christopher W. Kahler, director of the Center for Alcohol and Addiction Studies at the Brown University School of Public Health. "Some of those involve being completely abstinent, and some of them don't."

Dr. Melissa Weimer, an associate professor who specializes in addiction medicine at the Yale School of Medicine, said that alcohol use disorder is a chronic disease. Those who reacted strongly to Mr. Pitt's account may have watched efforts to drink in moderation fail, in themselves or in someone they love, she added.

But treatment does not come in one size, she said. She presses instead for individualized plans, including medication such as naltrexone, which dulls the reward of a drink and helps some people cut back.

"We do recommend that people stop drinking entirely and probably continue to stop drinking once they have an alcohol use disorder," Dr. Weimer said. "But everybody's different, and they have to make their own personal choices."

Dr. Alta DeRoo, the chief medical officer of the Hazelden Betty Ford Foundation, the largest nonprofit substance use disorder and mental health treatment facility in the United States, goes further.

Dr. DeRoo said she refuses to shame or police how anyone pursues recovery, and she rejects the idea of a single correct route. Demanding abstinence and nothing else, she said, drives away the people who are still trying to quit.

Clinicians should meet patients where they stand, whether that means abstinence, moderation or harm reduction, walking beside them rather than grading them, she said.

Even when someone relapses, or tests moderation after a severe disorder, the sober years still count, Dr. DeRoo said, contrary to some conventional wisdom on addiction. Those people gained coping skills, repaired relationships and improved physical health, she said, and none of that reverses.

She reads Mr. Pitt's account as a useful one: sobriety, a return to drinking, a recognition that it was going badly, a correction.

“Sometimes it takes several times,” Dr. DeRoo said. “Right now it may just be harm reduction, and that’s what’s working for him, and if it gets him to a better place, I would support anything that helps a patient decrease their alcohol use and live a healthier life.”

Researchers differ on much, but not on this. Recovery, they said, confers no permanent status and has no finish line but rather what a person finds workable now, and whether it holds.

“What someone finds as recovery today may not be where they find themselves in five years,” Dr. Kahler said.

Mark Walker is a Times reporter who covers breaking news and culture.